

POLYP DETECTION MEDICAL REPORT

Analysis Date: June 04, 2026

VIDEO INFORMATION

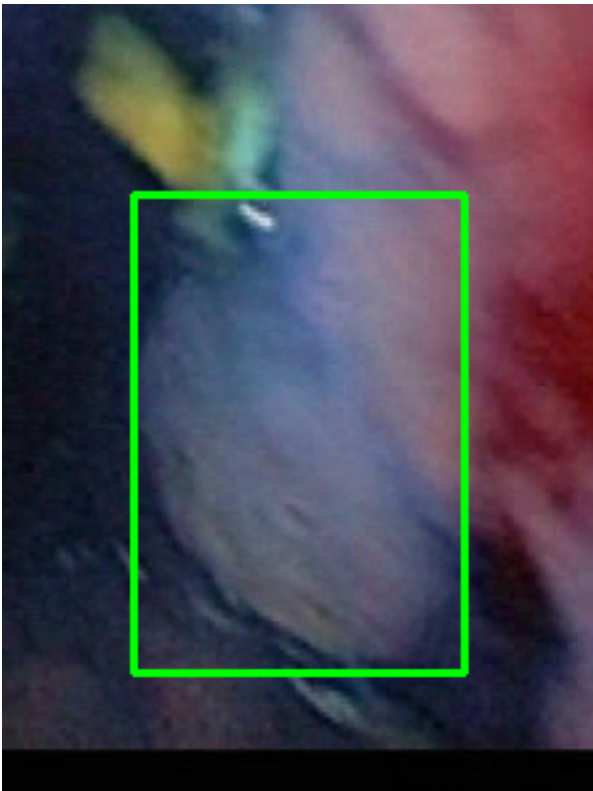
Video ID:	b0024c29-1d51-48f9-81ba-afd01d962cae
Total Frames:	777
Frame Rate:	25.0 fps (assumed)
Duration:	31.1 seconds

DETECTION SUMMARY

Detection Method	Count	Status
YOLO Detections	46	✓
RT-DETR Detections	280	✓
MedSAM2 Detections	249	✓
Consensus (3-Model Agreement)	3	✓
Risk Classification	Count	Percentage
HIGH RISK	0	0.0%
MEDIUM RISK	0	0.0%
LOW RISK	0	0.0%
TOTAL ANALYZED	3	100.0%

DETAILED POLYP ANALYSIS

POLYP #1 — SERRATED_POLYP [MEDIUM_RISK] (Tracked across 53 frames)



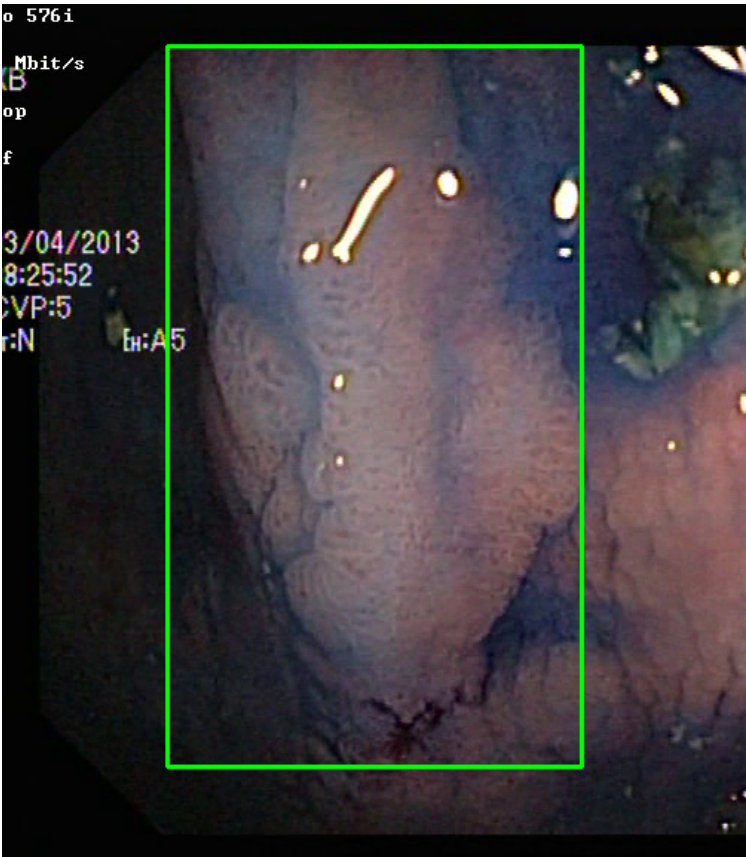
Feature	Value	Interpretation
Frame Index	35	Frame 35 of 777
Detection Method	Detector Consensus (MedSAM2 Unavailable)	MedSAM2 Unavailable (MedSAM2 unavailable for this video)
POLYP TYPE	SERRATED_POLYP	Type confidence: 73.8%
Redness Score	0.097	Color-based vascularization
Relative Radius	58.0%	Polyp size as % of frame diagonal
Texture Score	0.065	Surface morphology
Vessel Visibility	0.175	Vascularization indicator
Risk Tier	MEDIUM RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	LOW_RISK	Final symbolic reasoning bucket

Expert Prediction	LOW_RISK	Decision Tree output
Clinical Class	NORMAL_MUC OSA	Normal mucosal pattern — no polyp morphology identified
Detection Confidence	70.4%	YOLO / RT-DETR / track confidence

Type Assessment: Serrated lesion — consider serrated polyposis syndrome. Complete resection required.

Low-confidence MODERATE RISK detection - Clinical review recommended

POLYP #2 — LIFTED_POLYP [LOW_RISK] (Tracked across 29 frames)



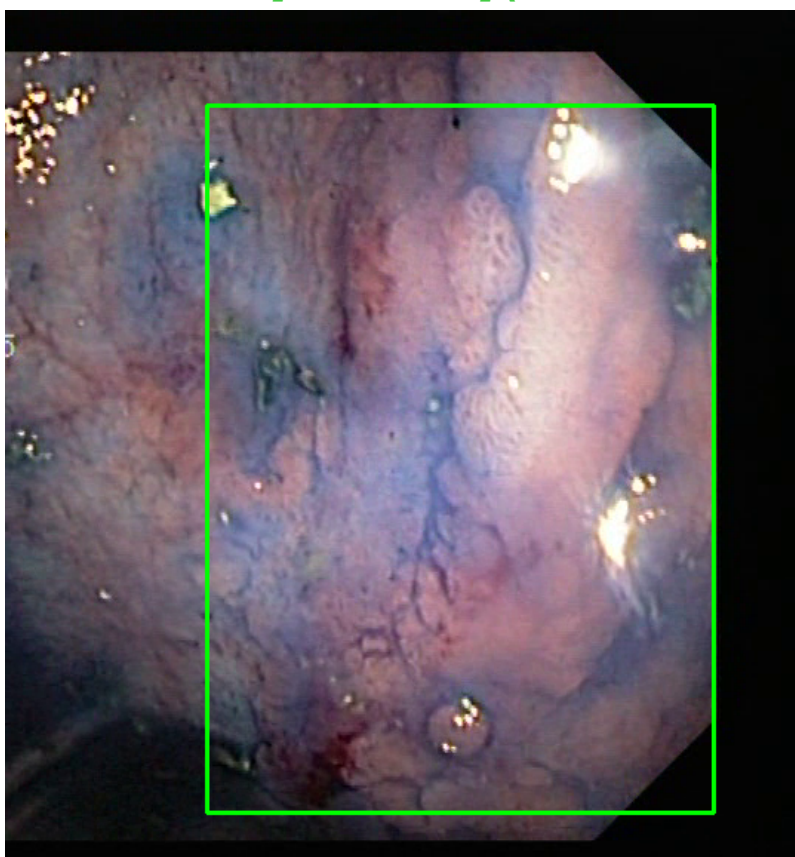
Feature	Value	Interpretation
Frame Index	296	Frame 296 of 777
Detection Method	Detector Consensus (YOLO / RT-DETR)	Model Agreement (MedSAM2 unavailable for this video)
POLYP TYPE	LIFTED_POLYP	Type confidence: 59.3%
Redness Score	0.045	Color-based vascularization

Relative Radius	69.2%	Polyp size as % of frame diagonal
Texture Score	0.076	Surface morphology
Vessel Visibility	0.001	Vascularization indicator
Risk Tier	LOW RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket
Expert Prediction	HIGH_RISK	Decision Tree output
Clinical Class	NORMAL_MUC OSA	Normal mucosal pattern — no polyp morphology identified
Detection Confidence	66.2%	YOLO / RT-DETR / track confidence

Type Assessment: Lifted lesion post-injection — submucosal plane confirmed. Resection feasible.

Low-confidence LOW RISK detection - Follow standard protocol

POLYP #3 — LIFTED_POLYP [LOW_RISK] (Tracked across 21 frames)



Feature	Value	Interpretation
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Frame Index	233	Frame 233 of 777
Detection Method	Detector Consensus (MedSAM2 Unavailable)	MedSAM2 Unavailable (MedSAM2 unavailable for this video)
POLYP TYPE	LIFTED_POLYP	Type confidence: 81.2%
Redness Score	0.072	Color-based vascularization
Relative Radius	5.2%	Polyp size as % of frame diagonal
Texture Score	0.568	Surface morphology
Vessel Visibility	0.024	Vascularization indicator
Risk Tier	LOW RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket
Expert Prediction	HIGH_RISK	Decision Tree output
Clinical Class	ADENOMATOUS_POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	73.7%	YOLO / RT-DETR / track confidence

Type Assessment: Lifted lesion post-injection — submucosal plane confirmed. Resection feasible.

Low-confidence LOW RISK detection - Follow standard protocol

CLINICAL IMPRESSION

Summary:

A total of 3 polyps were detected and analyzed using multi-model consensus and AI-based risk stratification.

Risk Distribution:

- HIGH RISK polyps: 0 (0.0%)
- MEDIUM RISK polyps: 0 (0.0%)
- LOW RISK polyps: 0 (0.0%)

Recommendation:

All detected polyps are classified as LOW RISK. Standard surveillance protocol is recommended.

Technical Details:

- Detection: Multi-model consensus (YOLO, RT-DETR, MedSAM2)
- Features: 444-dimensional vectors (384 SSL + 60 biomarkers)
- Classification: Cluster-specific Decision Tree experts
- Confidence: Based on deep learning + medical heuristics

Disclaimer: This report is generated by an AI-assisted research system for academic and research purposes only. All clinical recommendations are derived from published ESGE 2017, NICE CG118/CG131, and BSG 2019 colonoscopy guidelines. This output does not constitute medical advice and must not be used as the sole basis for clinical decisions. All findings must be reviewed and confirmed by a qualified gastroenterologist or endoscopist.